

Tue, Dec 06, 2022 9:17AM • 56:33

Interviewer 00:00

Perfect. All right. Let me see if Oh, okay. Yeah. No technical issues there. Yeah. All right.

Therapist 00:08

Okay. Oh, continue. I must say oh, probably Yeah, right. Okay. Yeah. Okay.

Interviewer 00:13

Should sorry. Should I call you ...or?

Therapist 00:

Yeah, well, In Deutch It is... But yeah.

Interviewer 00:21

All right. And just to start with, I just want to get a better sense of your clinical practice. You're currently in the [Country 1]. Right? Yeah. And you're based in the [Country 1] and you provide services to [Nationality 1] couples.

Therapist 00:34

in general. Yeah. Sometimes also expats. So I do have, you know, or people from [Country 2] sometimes because I also speak [Nationality 2] fluently, but generally, I mean, [Continent], let's say that, yeah.

Interviewer 00:49

All right, perfect. And your primary modality is what kind of couples therapy do you primarily provide to couples?

Therapist 00:57

Oh, well, that's a difficult, that's the most difficult question right away, because I, I wouldn't call myself that I am into a certain paradigm. I work very organic. So I combine, it's very eclectic, I combined a lot of stuff what I do, like, is the act. I call it approach. So So I that's one of the bases as well. And sometimes it's more cognitive based, sometimes it's more emotionally focused. So sometimes, you know, it really depends very much on here, what's what's being presented in front of me. So that was also my first thought, thinking like, okay, okay, okay, this is again, this this thing about developing this, this scale, and I don't work with scales. But I can imagine it might be helpful sometimes.

Interviewer 02:08

Perfect, And, we'll get there. But before getting there, do you specialize in certain types of couples that you provide?

Therapist 02:16

Um, well, I think in general, I have a quite an average mainstream type of couples generally heterosexuals. But homosexuals do occur, generally monogamous relationships, but open relationship based couples do occur? So I think the main or the main profile of my clients is generally white. [Continent] again, I mean, it's. So that would be culturally, a bit, the setting. But then in age, it ranges from quite young couples, to couples who are really far in their [70-75]s Already, sometimes even. So it's, yeah, but it's quite mainstream. Yeah.

Interviewer 03:19

And how long have you been working as a couple of therapy now?

Therapist 03:24

I would say, when really got a boost. I think that would be like [6-10] years ago, maybe? Like that. Yeah, yeah, I've seen a lot. Yeah.

Interviewer 03:35

Right. And again, just to get a better sense of your practice, I wanted to know, when your clients come to you, what is usually the presenting issue, or the reason that they seek your service? And I know there's gonna be a range?

Therapist 03:48

Well, yeah, there's a broad range. I mean, the most common one complaint, reason why they come is either having grown apart, having lost a connection and not feeling okay, anymore, and seeking that connection in which to be honest, I find that mostly women seek more the emotional connection, and many men seek mostly sex. I mean, that is, I think, very typical, but this is how I think it is. Or when there is infidelity going on, or has been detected, things like that. And also what I do specialize, because I in because I also specialized in that in my other, I mean, when I work with individuals as well, I work a lot with people with ADHD on the spectrum of ADHD and sometimes on the spectrum of autism. And I find that actually, far more people than one might suspect are somewhere on that spectrum. And I find also that very many couples that seek help, especially when it is about the connection part, there is something going on there. So more, quite often I do detect that, especially one, mostly the men, you know, might be somewhere on the spectrum, or at least finishing against it a bit. So that this, this issue about being able to, to communicate and reciprocity and everything is at stake. So I find that a lot, actually

Interviewer 05:33

super interesting, because

Therapist 05:37

they are very, that's the thing, what I really see is what I mean, when I found that out, when I noticed that in the beginning, I was a bit surprised and says, like, Oh, that's funny. But then I realized that this is very common, how it works, that people who are on the spectrum of autism, having their lacks, you know, lacking social skills, they are very attracted to the more bubbly, you know, happy go lucky type of partner, and her generally being able to lift him up a bit in that part, the person on the ADHD spectrum, who is quite a bit sometimes unstable, and is seeking for that calm, stable, you know, type of person, or

they do they are attracted. But then along the way, I mean, reality kicks in and then problems occur. Yeah,

Interviewer 06:40

that's super interesting, because I'm more familiar with the therapeutic approaches to handling conflict, for example, because with other relationships, therapists as well, that's their, usually the presenting case that they don't get anymore of communicate badly always leads to fighting, but having loss of connection is less familiar to me. Like, can you give me like a briefer, for example, for what that would look like, of regaining this connection for for the couples in couples therapy?

Therapist 07:13

Yeah, well, I mean, what would be typical, if I would have a couple presenting that problem. And if, for example, if there would be autism in the somewhere in the game, what you would see is that the I mean, most of the time, it is the man who is more in the autistic range, and the woman is more on the ADHD range, you know, or at least when there is one autistic, and then the non autistic partner will be, especially when kids come along, because this is really a very big game changer. When kids come along, this partner tries to, to get that her spouse to, to to to interact with the kids and see in the house, what's going on what should be done. And you see that when all the with the hustle and bustle of kids and the whole, you know that that the more autistic person tends to draw. So either they start working a lot, or being upstairs in their study, or a lot of gaming, a lot of screen watching being on the tablet being on the phone being on. So being and not being able to, even to notice what is necessary, you know, to what should be done in the private situation in the home situation. And not also very often not being able to communicate about what is going on inside because I see that with autistic people that they do have difficulty even to notice, and to give words to what they feel they are they don't even make a connection. I mean, they can be tense and they can be anxious, but they don't notice that they are because that is also one of the issues that you find in autism. But of course, they show it and their partner sees it. So she goes what's going on in your mind, oh, nothing is going on. I want all the typical thing there. And that's okay for once or twice or three times. But if that happens, time after time, over time time, if she starts pushing more and he withdraws more, or she stops pushing and he withdraws and keeps himself there. So there you find very often that that there is loss of feeling and what I feel is what also very often combined is combined with that is that what I really do think a very classical Typical male female heterosexual dynamics is that women too in order to have sex so to speak, they need a feeling of connection they need the feeling of her husband asking How was your day honey and saying Oh That dress looks nice on you or whatever, you know, men they seek much more for sex and find that and that is for them intimacy and also being appreciated in their manlihood and in their you know ever. So, you see there on top of that this this issue coming because she will she doesn't feel like like sleeping with him and he does regardless whether they are apart when he you know, because it doesn't matter so much for him. So, long that so, that I find is actually the dynamics of two quite complex steps of dynamics that interact and give another whole cocktail so to speak. Yeah,

Interviewer 11:13

Interesting So, you're like orchestra leader of a complicated very complicated piece. And sometimes I like to ask as well you mentioned that you do not like scales, I was wondering how you did the assessment of the couple's how to for example, diagnose their problem, so to speak.

Therapist 11:39

they do get I do send an intake form and I just looked prior to this meeting to my intake form. So what is it actually I am asking and what I generally what I do ask is how they see their the problem that you have so I let the I let both partners to fill it in separately hopefully they read it each others but not some not always. So I let them describe what they feel is the problem because I think that is the most important thing how they look at not what what I think is the problem I want to know what they think is the problem and from there on I ask what their history is as a couple how they get a couple of become a couple How long have they been together whether there are any issues that rose to seeking help for example infidelity or financial problems or children or death or illness or whatever see your so I mentioned all these possibilities as you know so they can write that down so they're there their history as a couple is is to me very important to to see okay what what might be going on there. So up till which point Did everything go well and when came trouble in paradise, you know? And then I also asked a lot about their personal situation from what background in what background did they grow up what was their family situation when they grew up? How was their relationship to their parents what are what kind of work are they do you can find a lot of information out of what kind of work people are doing as well what professionals they have illnesses physical illnesses, mental illnesses that have been maybe playing a role drug or alcohol how use or abuse so, that is all kind of that's all background information that I do ask them before they even come to me. So when they come in I have a quite extended view already actually on important parts in their lives. And from there on, I can distill certain possible conflict issues or certain issues that might play a role in in the problems that they are having together.

Interviewer 14:32

Interesting and then this intake form – do you know how long it usually takes for them to fill this form?

Therapist 14:39

no no. And that is also because it is an open form they can write down as much as they want as little as they want and it's very you know, generally women they really right light and men say or connection six or you know bit like that, so that's fine, you know, I'm okay with that but so it differs very much. And sometimes people do say, Whoa, that really is thing. And so, you know, occasionally people do complain like, oh my god, this or that, you know, but generally people actually noticed that to them, it's helpful. Because I also asked them to link certain issues that I have written down are there any things that you have written down in what you're seeing, that might be playing a role in, in the prompts that you are now perceiving so I put them already at work, prior to the first session, just be okay, so what is in me

Interviewer 15:45

I'd like to explore a little bit more about your your opinion about scales, you said that you don't like them, have you know, them before and found them for example, is that why do you not like,

Therapist 16:00

why? Um, because probably, because it is so. So, objective that let us see. Human me, it can be interesting that there is a difference between person A and person B on how, for example, satisfied they are with let's say, finances, or sex or household issue, whatever. Sure. To me, it's not so very, very

interesting to see whether one person gives it a two and the one gives it the four, or, you know, sure, you can see whether it's a wide gap. To me, it's much more important to note that they do feel that that they are not satisfied in certain areas, for example. And to me also, actually, the whole scale bit is actually even disturbing the, the process of what's going on in my, my office, I'm dealing with people, I'm not dealing with numbers, and I'm dealing with feelings. Yeah, I didn't know it's a good question what it is about that, that's that it's It feels very artificial. In a very organic setting, I mean, you are two people and they feel miserable. And for a number of reasons, and sometimes these reasons are pretty much the same. And sometimes they are very different. And I always think, why not just ask them why not just ask them what it is that they why they feel so miserable in their relationship? What is going on? And why not just sometimes I even asked them Okay, so what do you think that that should change? Or you know, what, what would you suggest? I mean, what would be necessary to happen for you to feel okay, again, with a partner and, and I work from that, you know, I mean, I don't I it's not interesting to me how high and low and no.

Interviewer 18:42

In the history of your practice, you've never used these kinds of skills.

Therapist 18:47

No, I you know, to be honest, I do it to sound professional to into you know, like, Okay, it looks very nice and you know, impressive if you have this little I have one little scale down you know, at the bottom of my my intake form and that's that I took it from a book that I took of a course that I took, you know, so Okay, yeah, and for the rest actually. Yeah.

Interviewer 19:14

That's interesting. What is that small little portion? What was it about?

Therapist 19:20

This is very this is actually mainly the stuff that that I think that would occur in again, mainstream households. It's about financials it's about sex it's about appearances about religion it is about I can I can I just want I can call it up here I think does it let me hold on I can. So it is financials children and raising them. expression of love sex work career household, rust and jealousy in law's free time a letter drags out all religion, temperament and, and mood and goals and priorities and behavior and appearance

Interviewer 20:25

is that satisfaction, how satisfied they are with?

Therapist 20:31

This is how satisfied they are. And it ranges from a scale from one to seven.

Interviewer 20:37

All right. Perfect. Okay,.....

Therapist 20:43

So people do and I mean, I mean, at a glance, it might give an overall idea, you know, which is okay. But yeah, for the rest, it's it really is not very important to me. Yeah,

Interviewer 20:58

Interesting. All right. And I wanted to ask you, across the years, like different therapists, and different researchers have characterized the construct of relationship quality quite differently from each other. But I wanted to get your view about it. For you what is relationship quality?

Therapist 21:18

Yeah, yeah. Again, it is a very good question. I think it really is. It starts with this really subjective feeling, you know, what, what, what? What score? Would you give it on a scale of one to 10? I sometimes ask them, you know, what is it? And then I would ask them, okay, what what is that about, and very often for I think there's really difference in men and women here, really, because what, what they both very often do need is the feeling of appreciation being being appreciated for who they are, what they do, being respected for who they are, and what they do. So that is a very important thing. Also how they feel how they are taken seriously. And that is the thing, but then the big differences come between men and women very often is that for women, when they have the feeling, I am really happy in my relationship, it is when they do feel that connection with their partner, the sense of spending time together, having a good conversation, being noticed, because it is very important for women very often it sounds very, call it cliché, you know, but the typical things like they went to the hairdresser's and their husband doesn't notice she bought a nice little nice dress, he doesn't know this, or she's just wearing that nice dress. Even if he said before early, that it was a nice dress, she wears it again, or she lost a couple of pounds, and he doesn't notice or he does notice and he doesn't say you know all these things, that is for women, extremely important. And for women wants their children because this is also I mean, whether there are children or there are not children is a very big game changer. And for women, generally, I always tell men, there's one thing you can make a woman happy with when you are a father and that is by voluntarily changing your kid's diaper. And taking out the trash without her asking you and getting your eyes out of your pockets and noticing that, yes, it the house needs some vacuuming. And they use pick it up and you do it without her asking if this is a very common thing. And it sounds very profane. It goes very deep because it goes to the point that women say I feel I have another child in a grown up suit. And I don't want to be I mean, these things need to be done. The child needs to be trained to child needs to be fed, the child needs to be whatever. And I know that if I asked him, he probably will do it. I don't want to ask him. So they feel very often that it becomes a very an equal relationship of a top down and men feel also very often that she starts nagging. So that is a very common dynamic that occurs, especially with young children around.

Interviewer 25:17

Interesting.

Therapist 25:18

So this inequality. Yeah, and men generally, they very often mentioned how they feel not appreciated, for example, fact that they have been working their butts off for trying to get a promotion making over time. Because that very many, many men do have this very classical, I mean, it's very classical very often, this idea of the providing, and that they feel the responsibility, even if the woman is also having a

job, very often this feeling that I need to be providing for my family. And that what they put in that, that this is not appreciated often enough. So when they come up with a beautiful diamond ring, very often a woman can say, Oh, that's easy, you can go to a jeweler. And buy a nice ring for me, but that's not going to change things between us because that's not going to make us connected. For him, that is this thing like oh my god, you know, you don't know how many extra hours I work to afford that ring. So also, so this concept in women and men on what their input is, in a relationship is is vastly different.

Interviewer 26:45

Interesting. So for you, what differentiates couple of high relationship quality and those with a low relationship quality.

Therapist 26:55

Um, I've been thinking about that what I discussed this morning with a friend, what I feel is that when people are able, that's my daughter, I'm sorry, she was sitting next to me and she has something when people are able to, to, I'm looking for the English person to look at their own contribution to where things go wrong, we put whether you can put a hand in your heart and say, Okay, I was wrong here or I could have done that differently. So whether people are willing and able to step in the shoes of the of their partner and imagine how it could be for them to live with a partner like they are for example. So whether they can see their own contribution in the negative sense to what is going wrong, the in the relationship, and whether they are able and willing to acknowledge that maybe they made a boo boo, maybe they made a mistake, maybe they weren't so nice this time maybe whether they are able to say sorry, whether they are so this, this quality, I think actually when it goes to prediction of healing, when this this couple will make it that is that is so big. If that's not there, I know it's not gonna work.

Interviewer 28:41

Interesting. And there are some people who would make the distinctions as well and not sure if you will, that there are for example, these kinds of behaviors that are that would make a relationship survive. And then there are I don't know maybe behaviors or thoughts that make our relationship flourish. Like for you these types of like oh, you should be able to take you know, just step into the shoes of your partner you should be able to look on heart and see

Therapist 29:11

It is a question, i really asked very often to my my clients imagine I wouldn't be for you to live with the you your wife having the characteristics that you have. And I also asked very often so you know by now all the things that she's has been nagging about for example, you know, so just try to imagine would there be at least a few in there in which you would be able to admit that actually she's she's right, that it is what you call nagging that is it really so unreal is it so unrealistic. So, you know? And and if you think yes, it really is not realistic. You know, I asked them to explain why why they think it's, you know, so they need to work on that idea. And sometimes they can explain why indeed, it really is nagging. And sometimes they they can come to the point of saying, okay, yeah, well, actually, yeah, well, yeah, she she simply is right, you know, that might be painful, but helping? Yeah.

Interviewer 30:36

And are there things? Are there things that couples can do, aside from? Let me try to articulate this because while you were talking as well, because I'm thinking, are there things that they can do to make the relationship not just survive, but to flourish? Aside from taking aside from what you're talking about, aside from making their own? Like, being accountable for their own contribution and where things go wrong in their relationship? Do you make that distinction?

Therapist 31:10

Yeah, well, the thing is sometimes it really depends also on I mean, of course, you know, it wouldn't be ideal if if a couple could make the relationship flourish. I actually think that sometimes making it to survive is already can be sometimes even given the circumstances enough sometimes I have couples of which I am pretty sure that once the kids grow up and once they are out of the house, they probably will split up and because they have a very strong wish to have their children grow up at least in a family setting with a mom and a dad in one house and not you know, part everything you know, sometimes it is the goal is making it to survive. But if there is this need to flourish indeed you know, I always I start with finding out so how did you become a couple in the first place? So what was there that made you flourish? You know, but also was there flourishing because sometimes you find that a couple was already from the start you know, that the quality may and also here comes another issue is that is on how well he called the hefting attachment how safely or stay attached they were as a child, so for some people, when they when they become when people become a couple, when there's a lot of issues, they're in being afraid of being left alone or you know, then sometimes the relationship isn't good from the start and there wasn't flourishing already back then. You know, so then and of course then I sometimes do talk to them about you know, what it would mean to them maybe to split up that this might be a good thing but but go with the flourishing Yeah, going back to what what was it that that you were I very often also really bring back to life that moment of when they fell in love What was the moment they thought oh my gosh, she's the one he's the one he was so Oh, she was so oh my god, what is still there? Oh, those are eyes. I can still meld what he does. And that's fun because then you see their faces lighting up to oh my god is really great look, you know, and you see things changing so I tell them also can you imagine how beautiful it would be if you would tell her more often that you still can sink way in her eyes and how beautiful our eyes oh, yeah, you know, so I I help them to to pick up on that as well. Try to do more things that they did. Sometimes it's simply it's not possible especially when I have people that met when they were traveling or that they did a lot of traveling and kids came again kids are a game changer really that is I can tell you you know.

Interviewer 34:36

And just to circle back as well on the topic of relationship quality, because we you said for example, sometimes you make them and say like oh subjective feeling of one to 10 and then the way that men and women are similar and the way that they are different. So for you would it be correct to say that that for relationship quality for you is really the subjective feeling of these individuals about how they feel about The relationship?

Therapist 35:01

Yeah. And of course, I think it is governed by a certain amount of fixed issues that you find in most couples, I'm sure. And then also, depending on whether they're male or female, because I really do

think that whether we want or not, I think that is a very major influence our sex and but then, within that range, it can vary tremendously.

Interviewer 35:40

Interesting, and how that's how you conceptualize relationships often differ from how your clients conceptualize relationships?

Therapist 35:56

I try to, to keep my own conceptualization in the background, because it is not my life, I mean, I can have this idea about what would be the ideal relationship. And it might match with their idea, but it might not as well. So I really try to focus on them, what is important to them? Because I really think that is what it is about, and, and it should work for them. Not for me, not for my neighbor. Just them.

Interviewer 36:40

And we also know that the relationship is a complex system, it involves love, sex, relationship, maintenance, conflicts, satisfaction and etc, etc. During therapy, how do you decide which one to focus on? In this complex system, the target?

Therapist 36:58

On the issues where I see is the most emotion, most hurt the most frustration, the more intense, most intense is the most despair. That's what I focus on. The rest is, well, probably good enough or not bad enough to really, if I have to pick my battles. I focus on what makes people cry. Actually, yeah, yeah.

Interviewer 37:27

Okay, interesting. And how do you determine when to end the couples therapy,

Therapist 37:32

when they feel that they are ready. So I again, also I don't work with protocols, I don't have like, Okay, we have like, five sessions or 10 sessions. I also definitely don't work with Okay, today, we're going to do this. And next time, we're going to do that. Because I play by ear very much I pick up on, on what has been going on more often than not, I also noticed that even if I gave them kind of a, an exercise, or you know, some kind of homework, whatever, something else came up, which turned out to be much more important than that. Exercise. So I also am very vague in picking up on you know, on the next session, I just asked a very simple, okay, how did things go? And then they say, well, either say, oh, yeah, really, you know, so what went well, and sometimes it does relate to something that they they did for homework, but sometimes it is this whole different thing that came up. But that turned out to be very important. So, again, I work very Yeah...organically

Interviewer 39:01

I know, there was bound to be a broad range of it, but usually, how long does it take from start to finish of their couple of therapy?

Therapist 39:09

Oh, yeah. I mean, sometimes I really have nothing like for four or five sessions, sometimes it sometimes 10 I do have couples that are in some kind of ongoing thing. And that you also know that they just need some support to survive to survive and help it make a bit better, but we probably won't reach flourishing, but you know, good enough to be okay with it and not split up. But let's say between five to 10 sessions.

Interviewer 39:48

Interesting, and these are every week, every other week?

Therapist 39:52

Definitely not, every two weeks every two weeks now using every one. People have too much in their lives alone. school, work, kids. Yeah. And they need to, they also need time to reflect, they need time to things to sink in. And they need to find examples in their daily life that they can relate to like, Oh, this is what you meant what we were talking about, or this is typical, you know, so you need time is a very important factor as well.

Interviewer 40:29

All right, I probably know your answer to this, but I still need to ask, we're currently developing a cross cultural relationship quality scale that, you know, hopefully better presented relationship as a complex system better than previous scales that will help therapist and treatment planning and eventually, yeah, how interest would you be in the scale

Therapist 40:54

Really depends. It really depends. If it would be kind of a not too long, and we'll see if it would help me too easily pick out a few items in there that think oh, that might be interesting to focus on. That might be interesting to me. Or whether there would be some items on the scale that I think oh my god, that's interesting. I never thought of that even might be of influence you know, that would interest me definitely. But to be honest, for the rest, I mean

Interviewer 41:46

Okay, because I was asking me yeah but it gets this it's gonna be a two year thing we're planning to develop it and validated over the course of two years. Would you mind for example, because later on as we validated of growth, we would also love feedback from from the therapists that would potentially okay for you to contact you again.

Therapist 42:13

Yeah. Yeah, yeah.

Interviewer 42:14

Okay. I'm sorry, but I think I will need a couple more minutes after the time that we initially set

Therapist 42:27

no problem it is my day off, so it's okay. Don't worry.

Interviewer 42:30

All right. Now, before we end, I just like to go over our discussion that we've had well, we talked about your clinical practice you said that you practice in the [Country 1] mostly [Nationality 1] couple but for expats, also [Nationality 2], [Continent] in general. Your you have an acclaimed couple therapy practice it might be ACT, it might be EFT, you work organically, your clients are general mostly heterosexual, but also homosexual here and then also non monogamous here and that I wanted to ask about that, by the way, are the problems that they seek treatment for? are basically are there differences usually in how they in the therapies for for heterosexual phrases homosexual couples that you've encountered?

Therapist 43:25

Yeah, I think so. Very often in when I when I see male couples, very often, it can be a bit more around the sexual issues on fidelity, I find a lot more I mean, not everybody a lot more homosexuals tend to be a bit more promiscuous or had at least promiscuous background before they become became monogamous. So, sometimes issues about that also, sometimes issues about how it was ? or things like that. So that might be an issue. So, I see that more and in women it is much more about jealousy in the sense of Yeah. What much less when it is about for example also when they have children you know, then things generally work out a bit better than because they are two women, I think. But then it is more often up till now what I saw is things more about infidelity then and yeah, jealousy

Interviewer 45:00

Writing and for non monogamous for this monogamous couples? Are there differences that you would mark out between the two?

Therapist 45:11

Well, would they're very often this is when they are when they even if they have an open relationship that there is a difference in how open one would consider it and how open how open the other one would. So this is can be a very difficult issue as well, you know, and yeah, so that can be sometimes thing. Yeah.

Interviewer 45:40

Interesting. All right. And then we also discuss that when clients come to you with a presenting issue that they well that they present. You said the most common complaint was either growing apart having lots of connection, as well as infidelity. And he's also talked about, for example, autism spectrum, especially with children, that they tend to withdraw more and not being able to verbalize feeling and then that just feeds into the whole or losing connection, but also mentioned actually about ADHD. And I wanted to also ask that about a little how does that how does ADHD typically influence their behavior? When with autism, you said that they were more withdrawn, especially with when children come along, how does that play out with ADHD?

Therapist 46:29

Um, well, in the first place, what you see is when when, let's say, an autistic person, an ADHD person they meet, and when there are no children yet, you know, they can compensate each other's lacks of talents, and be a nice, nice couple in that sense. Once children do come, I mean, the the combination is

really difficult. But even if there's only one partner with for example, ADHD, and the other partner doesn't have that, and doesn't have autism, ADHD, people, they are generally they are chaotic. So they have difficulty in keeping their right now getting the bills paid, making the household work. And when there are no children around, it's not such a problem, especially when they have not been living together yet, it's not such a problem as your if your place is a mess, okay, sure, you know, I only need to stay the weekend there. And then I mean, my own place again, when they come together, you see that they are challenged, evolve their capacity in making things work so so Honey, would you pick up the that suit to a dry cleaner? Sure, and that person forgets, honey, you know, so all the stuff that they tend to forget the chaos, the problems with the very practical stuff with household but also people with ADHD, they have a tendency to be bored easily they are, you know, they are novelty seekers, they are risk seekers. And so they very often do have difficulty in keeping their interest in, for example, one partner, and spending the rest of their lives with that partner, so, but also in all the things that they do, careers are at stake, many job changes. So financial stability can be an issue that when that person is, you know, either fired again, because they have a lot of authority issues at work, you know, there's those typical things you see very much in the ADHD range, or they simply cannot get that promotion because this they, it doesn't they can't lift themselves up enough to do that. At the same time, their partner does expect something of them in that sense, you know, or very often also mood disorders. So we're talking also bit about kind of Psychiatry here, you know, mood disorders, depressions, alcohol abuse can be all found in that range. short tempered so especially also their you know, when they're stressed, especially with children, you know, yelling, really having meltdowns for for small reasons that can really influence the whole atmosphere, in the house in relationships. So this is one thing and then autism were they, you know, people with autism typically have difficulty with empathy. So they really have difficulty in just stepping into the shoes of the other, just imagining what does that person need. They really are very easily overwhelmed. So they need to was brought just to keep saying so to speak. And very often when children around again, are very, they are predictability seekers, they need predictability. Well, if there's one thing that's not predictable is when you have kids, especially young kids, a child with at three months of age has different needs than a child at six months of age or a year of it. So they have difficulty in kind of tuning in, in the needs of that trial, though, so they tend to, to, to stay behind with, you know, their wife told them, you know, you should do this with this with a baby. Yeah, sure. But by now, the child is six months old, and does different things than they were three months old, and they cannot lecture on on that. So you see, that is very difficult with autism, dealing with autism. disciplining the children also is can have an effect on that, you know, so that really can make things very difficult.

Interviewer 51:33

All right. Yeah, that's super super Interesting. All right. And then we talked about going back to the recap of our discussion earlier, as well, we talked about your assessment, your intake form, and how that plays into your clinical practice with their background, personal situation and your short scale of satisfaction with children, sex, etc. We also talked about why you did like scales about how it can be very artificial in a very organic, very human environment. And we also talked about your views on relationship quality, how, you know, it is for you, the subjective feeling, but also what the facts that are different for men and women, although there is similarity, the feeling of appreciation, and, you know, being seen for who they are, they respect that. But then, of course, it's very different in the sense that women are more more emotional connection, don't stand together, especially with children as well.

They don't want another child in the form of their spouse, or for men, it's also about being seen in the way that you might say, like, it's classical roles of traditional roles of mescaline, but still being appreciated for providing insight. And then you talk about what differentiates couples with higher relationship quality and lower relationship quality. And this is mostly about beyond being like looking through your own contribution, where things can go along, being really able to step into the shoes of your partner. And as you were saying, this will be difficult for people with other medical conditions and ASD, we also talked about survival and flourishing, and sometimes that survival is, you know, survival already, is

Therapist 53:26

it No, I mean, it can be a goal. Yeah, yes.

Interviewer 53:29

And flourishing, really is to, you know, to take them back to the start of a flourishing. Why do they flourish there? And really, when you go into therapy, the target is the issue with the most emotion with almost despair with the most intense and focus on that. And you'd like to play it by ear, you know, no protocols necessarily. And usually takes four to five sessions, 10 sessions or even ongoing for the people that yeah, survival. All right. Now, before we are and of course, the scale and you would potentially be interested in

Therapist 54:09

at least interested,

Interviewer 54:11

let me know how interested you are, but you find me and I also want to talk to you about how we will handle the data that we're getting here. Because of course with as a research project, we want to be as open and as transparent as you know, ethically and sensitively as possible. So the options for data sharing is one, either we can share the transcript portions that we have here, none of which can be identified to you. Or if you're a little bit more bold. Another option is to have the whole transcript, be publicly shareable, removed of sensitive and overly identifying information. we'll be anonymized.

Therapist 54:53

Me I mean, I have little to hide and I think you know what, what I'm telling you here is not something that I wouldn't tell anyone else or so if that's helpful is fine for me.

Interviewer 55:07

All right, so we'll go with option two. I'll be sending you a copy of your transcript if you want anything redacted. Let me know. Okay, and I think that ends our say, thank you so much. Great stuff, [Name]. Yeah, I will. I will not be a stranger. I will be needing to send you a transcript. Although it takes a month. Don't snitch on anyone. And later on after a couple of years when you hear back from you is a final version of scale but thank you so much.

Therapist 56:02

Well, you're welcome. And well, I just had fun and it's nice to also for me to really actually think of you know, so how do I liquid my profession here and yeah,

Interviewer 56:15

yeah, perfect. And as your granddaughter gets born into the world, I you know, my heart celebrates it. Yeah,

Therapist 56:21

I'm looking forward to it. Yeah, might be fun.

Interviewer 56:23

Yeah. Enjoy the rest of your day off.

Therapist 56:28

Thank you and enjoy your day as well. Okay.